PepSafe

FREE CHECKLIST

5 mistakes that cost you muscle on GLP-1 medicines

On semaglutide, tirzepatide or similar? In the big trials, roughly a quarter to around 40% of the weight lost was lean mass. These five habits make it worse — and each has a simple fix.

1 Eating as little as possible

The medicine already lowers your appetite. Very low intake makes it harder to keep muscle and leaves you tired and dizzy.

Fix: eat by the clock every 3–4 hours, to comfortable fullness.

2 Too little protein

Less food usually means less protein. A 2025 joint advisory on GLP-1 nutrition suggests a simple range of about 80–120 g a day (more conservative with kidney disease — ask your doctor).

Fix: about 25–30 g of protein at each main meal, eaten first.

3 Only doing cardio — or nothing

Walking is great for health, but muscle needs resistance. The same advisory recommends strength training at least 3 times a week.

Fix: 20 minutes at home — sit-to-stand, wall push-ups, bridges, backpack rows.

4 Not drinking enough

Smaller appetite often means less fluid too; with nausea or diarrhoea, dehydration can follow.

Fix: 6–8 glasses a day (NHS), sipped between meals; more if you're unwell.

5 Only watching the scale

Weight can fall while strength drops. That's not the goal.

Fix: every 4 weeks, count how many times you can stand up from a chair in 30 seconds. If it drops, review protein and training — and talk to your doctor.

Want the full plan? Protein targets, a 7-day meal plan, a 12-week home routine, a protein calculator and a printable tracker: **Protein & Muscle on GLP-1 — £4.99**. Free catalog of 200+ peptides: **pepsafe.info**

Sources: Mozaffarian D et al., Nutritional priorities to support GLP-1 therapy for obesity, Obesity 2025;33:1475–1503 · Look M et al., SURMOUNT-1 body composition, Diabetes Obes Metab 2025 · Wilding JPH et al., STEP 1, NEJM 2021 · NHS Eatwell Guide. Educational information only — not medical advice. Talk to your doctor before changing your diet or exercise, especially with kidney or heart disease.